

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Patient Name: Patient 15

Age: 62 years old | Sex: Male

MRN: 000-TEST-15

Admission Date: 06/12/2026 | Discharge Date: 06/19/2026

Attending Physician: Dr. M. Richardson, MD | Hospitalist Service

****PRIMARY AND SECONDARY DIAGNOSES****

1. Chronic Obstructive Pulmonary Disease (COPD) — GOLD Stage III (moderate), acute exacerbation
2. Peripheral Artery Disease (PAD), left lower extremity — stable
3. Hypertension — controlled
4. Hyperlipidemia — on statin therapy

****Hospital Course:****

Patient 15 presented to Metro Medical Center via ED with acute dyspnea, productive cough, and increased sputum purulence × 3 days. On admission, SpO₂ was 88% on room air; improved to 94–96% on 2L NC. ABG initially showed pH 7.31, pCO₂ 52, indicating mild respiratory acidosis. Spirometry performed on hospital day 2 demonstrated FEV₁/FVC ratio of 0.58 (predicted 0.70), consistent with moderate obstructive disease. Chest X-ray showed mild bronchial wall thickening without infiltrate or pneumothorax.

Treatment included:

- IV methylprednisolone 125 mg q.6h × 3 days, transitioned to oral prednisone taper
- Nebulized albuterol/ipratropium q.4h + PRN
- IV ceftriaxone + azithromycin × 5 days (sputum culture: normal flora)
- Supplemental O₂ weaned to 1L NC by discharge; SpO₂ consistently 92–95%

Patient demonstrated good response to therapy. Respiratory status improved; dyspnea on exertion (DOE) markedly reduced. Discharged in stable condition on optimized inhaler regimen and systemic corticosteroid taper. Peripheral pulses reassessed; no acute PAD progression noted. Patient tolerated ambulation without claudication symptoms.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Tiotropium (LAMA)	18 mcg	Once daily	Ongoing
Salmeterol/Fluticasone (LABA/ICS)	50/250 mcg	BID (morning/evening)	Ongoing
Prednisone (taper)	40 mg → 20 mg → 10 mg	Daily × 3 days each	9 days total
Albuterol MDI (rescue)	2 puffs	q.4–6h PRN dyspnea	Ongoing
Lisinopril	10 mg	q.d.	Ongoing
Atorvastatin	40 mg	q.h.s.	Ongoing
Aspirin	81 mg	q.d.	Ongoing

****FOLLOW-UP APPOINTMENTS****

- **Pulmonology Clinic**** — Dr. S. Patel, MD | 07/03/2026 (2 weeks post-discharge)
 - Review response to therapy; assess need for long-acting bronchodilator adjustment
 - Repeat spirometry in 6 weeks
- **Primary Care**** — in 1 week for BP check and prednisone taper monitoring
- **Vascular Surgery**** — in 4 weeks (routine PAD surveillance)

****ACTIVITY & WEIGHT-BEARING****

- Resume